جوستاف روسي انترناشيونال - مصر (EPE) *

طلب علاج أول مرة / متردد

إسم المريض رباعياً : سعاد سيد احمد مصطفي

المحافظة : القاهره **الحي / المدينة :** دار السلام **العنوان :** حدائق المعادي

رقم البطاقة : 26712110102346 **رقم التليفون :** 01142658211 **تاريخ تسجيل الإستمارة :** 18-10-2026

العمر(تاريخ الميلاد) : 11-12-1967 **المهنة :** ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحه-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي **المقر بما فيه**

التوقيع:(----------) **التوقيع:**(----------)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : علاج مبادرة سرطان الثدي علاج تدعيمي وتلطيفي [6579] **قسم العلاج :** الاورام

التعليق على التشخيص : ----------

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني **الوظيفة:** ---------- **التوقيع:** ----------

الاسم: د/ندى حسن **الوظيفة:** ---------- **التوقيع:** ----------

الاسم: د/نرمين رمضان **الوظيفة:** ---------- **التوقيع:** ----------

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة
رئيس اللجنة و مدير المستشفي.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفي قبل الصرف.

أعضاء اللجنة

ا.د/ ---------- **ا.د/** ---------- **د/** ---------- **د/** ----------

قرار اللجنة

التشخيص النهائي : ----------
خطة العلاج : ----------
المستشفي : ----------**قسم :** ----------
جهة العلاج : ----------**نفقات :** ----------
ملاحظات : ----------

رئيس	مدير عام	ختم
المجلس الطبي المتخصص	مديرية الشئون الصحية	شعار الجمهورية

Name: سعاد سيد احمد مصطفى
National ID: 26712110102346

تقرير طبي

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من	**بشأن تقرير طبي للمريض**
بقسم:	**رقم طبي:**

نتشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	**تاريخ وساعة الخروج :** PM 01:17 05-07-2026

وبعد الكشف والفحوصات والمتابعة وجد انة

المريضة تعاني من ورم غير حميد بالثدى و تحتاج الي **Supportive and palliative Treatment**

التاريخ : **05-07-2026**	**الوقت :** **-- --:--**
	الطبيب المعالج : د/شيماء لاشين





- هرمون
- دعيم (بكتل وخصيات

National ID · Front & Back
[[DOCTYPE:ID_FRONT_BACK]]

MDT form

Clinical data

Breast Imaging :

Path Workup :

cT4bN1M0 locally advanced pulmonary nodules 5 mm

DX	ER:	PR:	HER 2 neu	SISH :	Ki67 :
IDC	6/8	8/8	3		30

I	II	III
4b	3c	0

Sonomammography

- rt breast malignant UIQ

PATIENT HISTORY

pt received 18w weeks TC [illegible] cycles HP

POST HRH:IDC GII

free nipple , areola

III 1/13 , no extra capsular extension

free deep margin

partial pathological response

YPT1a YPN1

MDT OUTCOME & PLAN

For :

- POR1 (initially cT4b cN3c).
- Adjuvant T-DM1 x 12 cycles.
- Adjuvant AI for 5 years , then consider extended AI.
- Prophylactic zometa / 6 months.

SURGICAL ONCOLOGY

SIGNATURE

SIGNED BY

PATHOLOGY

MDT Form
[[DOCTYPE:MDT_FORM]]

Name: سعاد السيد احمد **MR. :** 7825	

Detailed Medical Report

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من		**بشأن تقرير طبي للمريض**	
بقسم:		**رقم طبي:**	

ننشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول :		**تاريخ وساعة الخروج :**	

وبعد الكشف والفحوصات والمتابعة وجد انه

Post-mastectomy breast cancer, clinical stage ct4b, N3, with a 2 cm IDC, ER 8/8, PR 8/8, HER2 3+, Ki-67 30%. The patient received trastuzumab on 30/09/2023, followed by a right mastectomy and reconstruction with a port. Adjuvant therapy included femara and Kadcyla, with a planned schedule of 9 months of Kadcyla (SM 9/2025). The patient is currently free for Zopmeta and has received three cycles of femara.

تحريرا في ,

التاريخ :	20-09-2026	**الوقت :**	

الطبيب المعالج : dina makram

يعتمد ,

Ministry Of Health
Specialized Medical Centers
Dar El Salam Cancer Hospital
Radiology Department

وزارة الصحة والسكان
إدارة المراكز الطبية المتخصصة
مستشفى أورام دار السلام
إدارة الأشعة

Patient Name: Souad Sayed Ahmed Moustafa	Date : 05/04/2023
Exam : C.T CHEST WITH CONTRAST	Age : 55 Year(s)
Medical ID:	32336

POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN & PELVIS REVEALED: -

A known case of right breast cancer.

1. *Chest:*

- Right breast LIQ irregular speculated mass lesion measuring 3.8 X 3.3 cm.
- Right axillary level I and II pathologically enlarged lymph nodes the largest measures 2 x 2.1 cm.
- Prominent right internal mammary lymph nodes measuring 0.8 cm.
- Bilateral few scattered pulmonary nodules, the largest is noted at apical segment of right lower lung lobe measuring 5 mm.
- Middle lobe medial segment partial collapse.
- No pathological hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Average cardiac size. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.

2. *Abdomen and pelvis:*

- The liver is mildly enlarged with homogenous CT texture. No definite focal lesions or contour irregularities.
- Patent homogenously enhancing portal vein and is main branches as well as its main tributaries.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Cholecystectomy with operative bed clips. No collection.
- Prominent porta hepatis lymph nodes, the largest measuring about 1.2 x 1.6 cm.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- No pathological abdominal or pelvic lymph nodes.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both supra-renals.

CT Scan
[[DOCTYPE:CT_SCAN]]

Presidential Program for Women's health

Name: Soad Sayed Ahmad

Date: 11/3/2023

Bilateral Full field Digital Mammography

Technique

Mediolateral oblique and CC views.

High resolution ultrasound

Detailed Findings

left breast

- Scattered fibroglandular mammary parenchyma (ACR B).
- No masses, asymmetric densities, distortion or suspicious clusters of microcalcifications.
- Ultrasound demonstrated no solid or cystic lesion.
- Normal skin/nipple/areolar complex.
- Preserved axillary LNs.

right breast

- Scattered fibroglandular mammary parenchyma (ACR B).
- UIQ ill defined density.
- Ultrasound demonstrated 3-4 o'clock zone A an ill defined spiculated mass lesion measuring 40x23.5mm 3 mm from skin with evidence of micro calcifications and evidence of ductal extension.
- retracted nipple
- pathological axillary LNs at level I and II(more than 3) largest is measuring 18.3X13mm (eccentric hilum and diffuse cortical thickness).

Conclusion:

-left breast: (BIRADS 2) adenosis

-Right breast: (BIRADS 4C) UIQ malignant looking mass lesion and ipsilateral pathological LN for biopsy

Dr. shaden mahmoud

د. شادن محمود
جمهورية مصر العربية
وزارة الصحة والسكان
البريد الإلكتروني: info@whpp.gov.eg

Mammogram
[[DOCTYPE:MAMMOGRAM]]

NASSER INSTITUTE
ONCOLOGY CENTER

NUCLEAR MEDICINE UNIT

Patient Name: Soad Sayed Ahmed *Date:27/5/2023*

^{99m}Tc-MDP BONE SCAN

Desired information : skeletal surveying to detect multiple lesions.

Technique: Anterior and posterior whole body scan performed 2 hours after I.V administration of 20 mCi ^{99m}Tc-MDP:

Report:.

- No definite evidence of hot or cold spots that would account for metastatic bone lesions.
- Degenertaive changes are noted in the dorso-lumbar vertebrae.
- Both kidneys are normally seen.

Conclusion:

No definite evidence of metastatic osseous deposits.

Thanks for reference

Nuclear Medicine Unit

Dr.Sherein Mahdy , MD

Bone Scan
[[DOCTYPE:BONE_SCAN]]

Patient Name	:الأستاذة / سعاد سيد احمد مصطفى	Request No.	:13001023
Patient No.	:26712110102346	Request Date	:30/09/2023
Path No.	:3330/23	Verification Date	:15/10/2023
Sex	:Female	Financial Category	:اورام دار السلام - ثقة الدولة
Age	:55 Y	Department	:Out Patient
Referreing Doc	:	Registered by	:.marwa mohamed youss

Pathology Report

Diagnosis:
Right breast cancer post NACT, MRM specimen:
Invasive duct carcinoma G II (<5% cellulaity, Max tumour diameter 0.1x0.1 cm).
* Free nipple and areola.
* Lymph nodes were positive in 1/13 with no extracapsular extension.
* Free deep margin.
* Partial pathological response to therapy.
- Stage: ypT1a, ypN1.
- RCB: 2.15, RCB class II .

Signature
Prof. Dr. Lubna Omar.
Dr. Heba Hassan.

Verified by :
DR.Menna Nabil

غرفة رقم (4) – الدور الأول – مبنى رقم (987) – كورنيش النيل – مصر القديمة – القاهرة – مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

Patient Name	: الأستاذه / سعاد سيد احمد مصطفى	Request No.	: 13001023
Patient No.	: 26712110102346	Request Date	: 30/09/2023
Path No.	: 3330/23	Verification Date	: 15/10/2023
Sex	: Female	Financial Category	: اورام دار السلام - فئة الدولة
Age	: 55 Y	Department	: Out Patient
Referreing Doc	:	Registered by	: .marwa mohamed youss

Pathology Report

Clinical Data: Right breast cancer, post NACT.

Nature Specimen: Right modified radical mastectomy specimen.

Gross Examination:
* Procedure: Right MRM measured 24x20x6 cm with covering skin ellipse measured 20x17 cm.
* Tumor extent: A greyish white firm tumor bed site at UIQ.
* Size: 2.5 x2 cm.
* Skin involvement: Nipple & areola were unremarkable grossly.
* Distance from margins: 2cm away from deep margin.
* Lymph nodes number and size: Axilla measured 10x8 cm: Axillary dissection entangling 13 nodules up to 1.5x1 cm.

Microscopic Examination:
* Histological type: Invasive duct carcinoma.
* Grade (differentiation+ nuclear pleomorphism+ mitosis): (3+2+1) Grade II.
- Maximal tumor diameter 0.1x0.1 cm
- Actual tumor cellularity: <5%, tumor necrosis index: 95%
* Ductal carcinoma in situ: No.
* Perineural invasion: No.
* Lymphovascular emboli: Occasional.
* Skin involvement: Free nipple and areola.
* Lymph node involvement: 1/13 with no extracapsular extension. lymph nodes showed therapy response.
* Resection Margins: Free deep margin.
* Histologic response to therapy: Partial pathological response.
* Microcalcifications: N/A.

Verified by :
DR.Menna Nabil

غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر الجديدة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

٧٨٢٥

Patient's Name: **Soad** **Said Ahmed Mostafa**

Date: **2026-09-06**
Referred by: Prof.Dr. **no doctor**
Hospital: **Goustave Roussey International Egypt**

ABDOMINO-PELVIC ULTRASOUND EXAMINATION REVEALED:

Liver: -

- *Average in size (15cm at Rt MCL) bright parenchymal echo pattern with smooth surface.*
- *No hepatic focal lesions or intra hepatic biliary radicle dilatation.*
- *Portal vein is patent and not dilated.*

Gall Bladder: -

- not visualized (contracted)
- *CBD is not dilated.*

Spleen: -

Average size , homogenous.

- **Kidneys: -**
- *Both are average in size,shape and echogenicity*
- *No stones or dilated pelvicalyceal system*
- *left parenchma hyper echoic lesion measures 10x9mm*

Para-aortic and pancreatic area*: show no gross abnormalities.*

No ascites

Urinary Bladder: *-*

- *Not distended enough however no stones or diverticular out pouching*

Uterus and adnexal *; no abnormalities could be detected*

Opinion:

Bright liver likely fatty
Left renal benign looking lesion likely angiomyolioma
Colonic gaseous distention

DR: Nermeen Ramadan.MD

٧٨٢٥

Patient's Name: **Soad** **Said Ahmed Mostafa**

Date: **2026-09-06**
Referred by: Prof.Dr. **no doctor**
Hospital: **Goustave Roussey International Egypt**

ABDOMINO-PELVIC ULTRASOUND EXAMINATION REVEALED:

Liver: -

- *Average in size (15cm at Rt MCL) bright parenchymal echo pattern with smooth surface.*
- *No hepatic focal lesions or intra hepatic biliary radicle dilatation.*
- *Portal vein is patent and not dilated.*

Gall Bladder: -

- not visualized (contracted)
- *CBD is not dilated.*

Spleen: -

Average size , homogenous.

- **Kidneys: -**
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Opinion:

Bright liver likely fatty
Left renal benign looking lesion likely angiomyolioma
Colonic gaseous distention

DR: Nermeen Ramadan.MD

[[DOCTYPE:MAMMOGRAM]]
Mammogram

NVCo

Medical No. : 9243
Patient's Name: Soad Said Ahmed Mostafa
Date: 2026-09-27
Referred by: Prof.Dr. no doctor
Hospital: Goustave Roussey International Egypt

جوستاف روسي انترناشونال مصر

Left Digital soft tissue mammography with high resolution sonography revealed:

Clinical summary: Status post right MRM & ALAND, followed by hormonal therapy for two years, routine follow up.

Findings:

Left breast.

- Scattered fibro-glandular densities. (ACR b)
- No speculations, masses, asymmetry, distortion or suspicious microcalcifications.
- **Ultrasound** demonstrated no solid or cystic lesions.
- Normal skin/nipple/areola complex.
- Non-specific axillary LNs.

Right mastectomy bed.

- Clear mastectomy bed with no masses or fluid collections..
- Clear right axilla. No pathological supra or infra-clavicular lymph nodes.
-

Conclusion:-

- ***Left breast (BIRADS 1): normal study.***
- ***Clear right mastectomy bed and axilla.***

Recommendation:-

- ***For annual follow up.***

Best Regards,

DR. Nehal Fouda,MD